



Arthrex ACP™ Double Syringe System

Autologous **C**onditioned **P**lasma



Biologics in Orthopedics

- Biologics has a solid history in Europe and some use in North America
 - Now gaining in popularity for orthopedic applications
 - Coining of the phrase “Regenerative Medicine”
- Why?
 - Increasing older population wants to maintain active lifestyle
 - We treat areas with limited blood supply and slow cell turnover
 - Greater demands on surgeons to produce more rapid return to sport



A Practical Approach

- Serious challenges for use of biologics in orthopedic applications
 - A system must:
 - Be affordable
 - Cannot add significant time to the procedure
 - Be easy to use
 - Corporate and Scientific Challenges
 - Regulatory control
 - Cost and time for development
 - Quantifying results

Our Answer - ACP

- **Autologous Conditioned Plasma**
- Platelet concentrate for use in orthopaedic applications
- Increased concentration of platelets and growth factors which are associated with the healing process



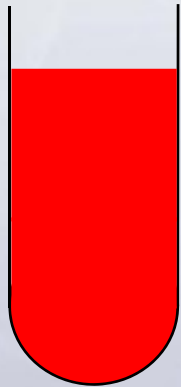
What Exactly Is ACP?

- An easy to use system that concentrates platelets and growth factors within a plasma layer **separate** from red and white blood cells
- Growth factors and other molecules within the plasma layer may modulate healing



What Does ACP Look Like?

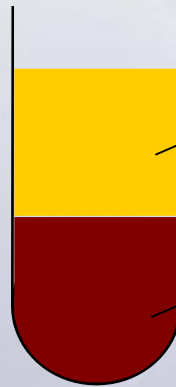
Blood + anticoagulant



Soft centrifugation



5 min / 1500 rpm (350g)



Platelet-containing
plasma (ACP)

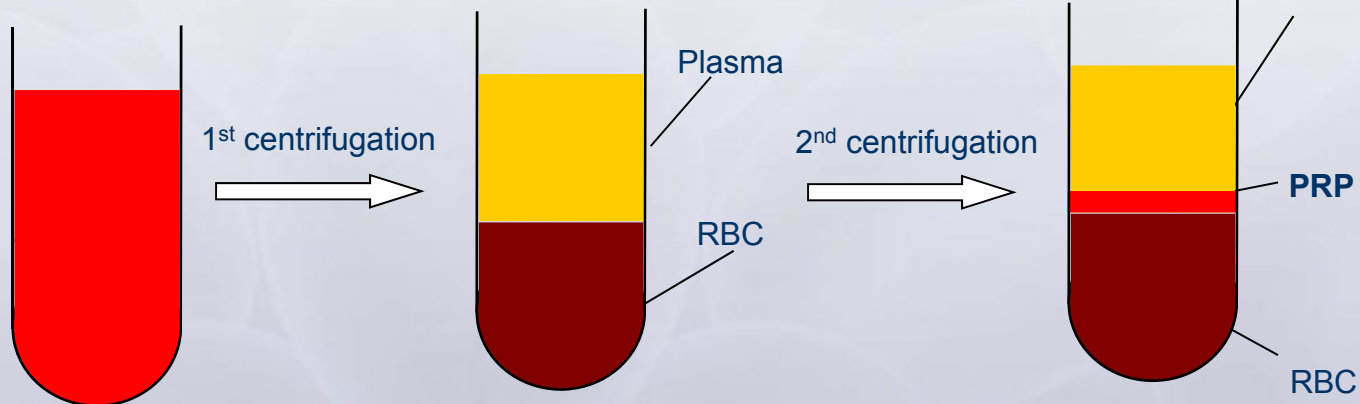
RBCs



- Easy system to use
- Minimal additional capital equipment
- Additional procedure time of ~5-10 minutes

What Do Other Systems Look Like?

Blood + anticoagulant



- Complicated systems that require rep support
- Expensive capital and expensive kits
- Additional procedure time (15 - 30 minutes)

FDA Clearance



DEPARTMENT OF HEALTH & HUMAN SERVICES

Public Health Service

U.S. Food & Drug Administration
1401, Broadway, Room 303
Baltimore, MD 21205-4128

December 18, 2008

Arthur, Inc. 000000

Attention: Maria Labarino
Regulatory Affairs Associate
1570 Chesapeake Boulevard
Naples, FL 34108

Re: (B)(5)(DPP)

Device Name: Arthur Double Syringe (ACP, Autologous Conditioned Serum System)
Regulation Number: 21 CFR 880.5850
Regulation Name: Platelet Syringe
Regulatory Class: Class II
Product Code: EMT and TDC
Date: September 18, 2008
Received: September 19, 2008

Maria M. Labarino

We have reviewed your Section 510(k) premarket notification of intent to market the device referenced above and have determined the device is substantially equivalent to the predicate device used in the enclosure (see legally marketed predicate devices marketed in interstate commerce prior to May 21, 1976; the enactment date of the Medical Device Amendments, or to devices that have been reclassified in accordance with the provisions of the Federal Food, Drug, and Cosmetic Act (Act) that do not require approval of a premarket approval application (PMA)). You may, therefore, market the device, subject to the general controls provisions of the Act. The general controls provisions of the Act include requirements for annual registration, listing of devices, good manufacturing practices, labeling, and prohibitions against misbranding and adulteration.

If your device is classified (see above) into either Class II (Special Controls) or Class III (PMA), it may be subject to such additional controls. A listing of such regulations affecting your device can be found in the Code of Federal Regulations, Title 21, Parts 801 to 898. In addition, FDA may publish further announcements concerning your device in the Federal Register.

Please be advised that FDA's issuance of a substantial equivalence determination does not mean that FDA has made a determination that your device complies with other requirements of the Act or any Federal statute and regulations administered by other Federal agencies. You must comply with all the Act's (21 CFR 801.60) and regulations, including but not limited to registration (21 CFR Part 807), labeling (21 CFR Part 801) good manufacturing practices requirements, as set

1 Indications for Use Form

Indications for Use

510(k) Number: BK074088

Device Name: Arthur Double Syringe (ACP System)

The Double Syringe (ACP) System is used to facilitate the safe and rapid preparation of autologous platelet-rich plasma (PRP) from a small sample of blood at the patient's point of care.

The PRP can be mixed with autograft and allograft bone prior to application to an orthopedic surgical site as deemed necessary by the clinical use requirements.

Prescription Use ☒ AND/OR Over-The-Counter Use ☐
(Per 21 CFR 801 Subpart D) (21 CFR 801 Subpart C)

(PLEASE DO NOT WRITE BELOW THIS LINE-CONTINUE ON ANOTHER PAGE IF NEEDED)

Continuation of CDER, Office of Cellular, Tissue and Gene Therapies (OCTGT)

Director, OCTGT

Christy R. [Signature], MD, MSc

510(k) Number BK074088

Using ACP in Medical Practice



Syringe, ACD-A, and cap



Withdraw blood + mix with ACD-A



Centrifuge 1500 rpm for 5 min



Transfer plasma into smaller syringe and unscrew



ACP ready to use

Using ACP in the OR



Syringe, ACD-A, cap, and sterile bucket



Scrub nurse withdraws blood + mixes with ACD-A



Scrub nurse places syringe into bucket and passes closed bucket to circulating nurse



ACP is transferred into sterile basin on sterile field. A sterile 5 cc syringe is used to recover ACP and use at the point of care.



Circulating nurse opens cap and scrub nurse removes sterile syringe



Centrifuge 1500 rpm for 5 min



Transfer plasma into smaller syringe and unscrew



ACP platelet concentration



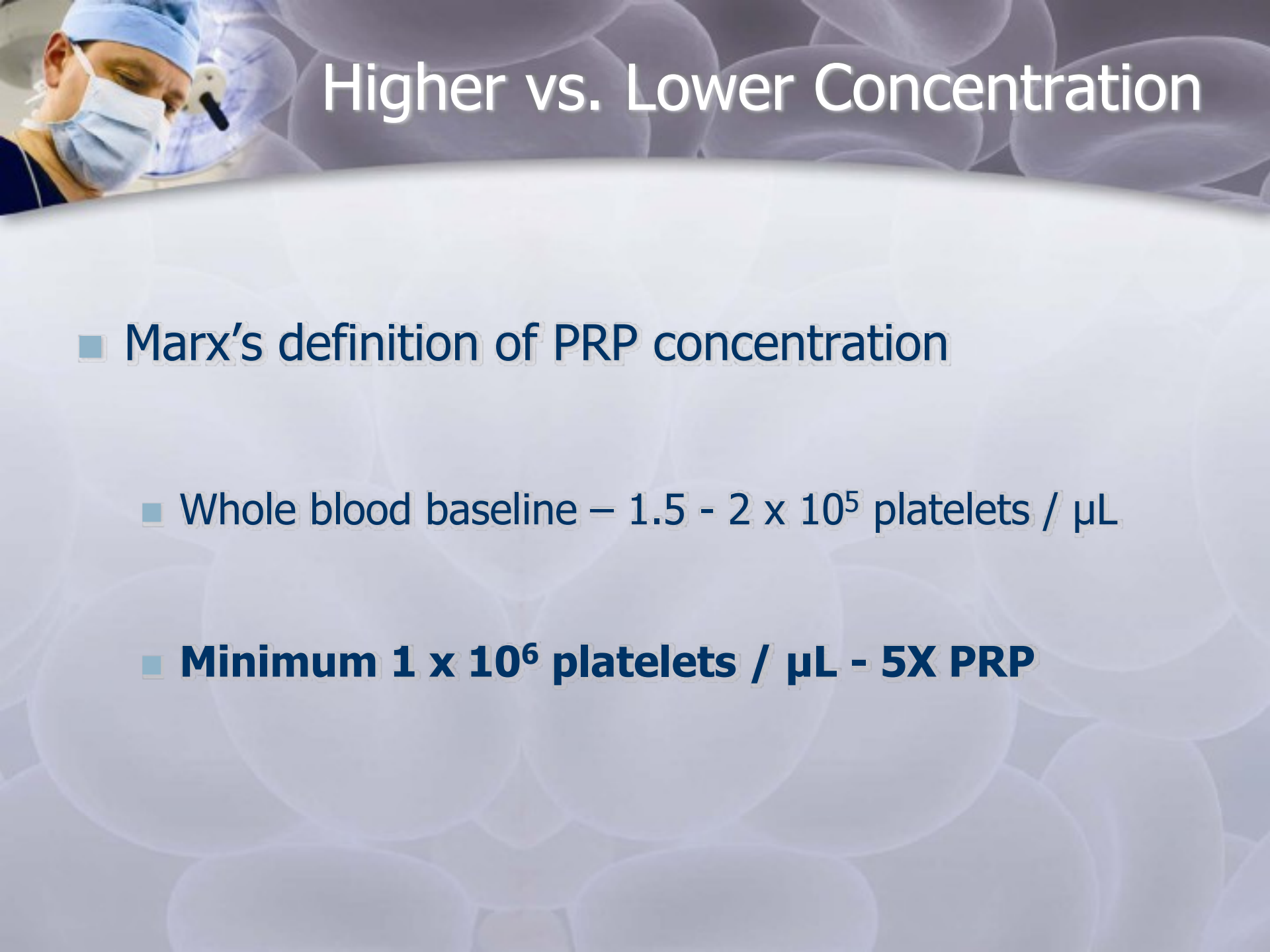
Higher vs. Lower Concentration

- PRP concentration is patient-specific
 - Pietrzak and Eppley, 2005; Weibrich *et al*, 2001; Anitua *et al*, 2004
 - Dependent on injury, patient condition, processing variables (single vs. double spin, PRP volume, etc.)
 - **Not necessary to have high concentration for effective healing**
 - **Higher GF concentrations can be detrimental to healing**



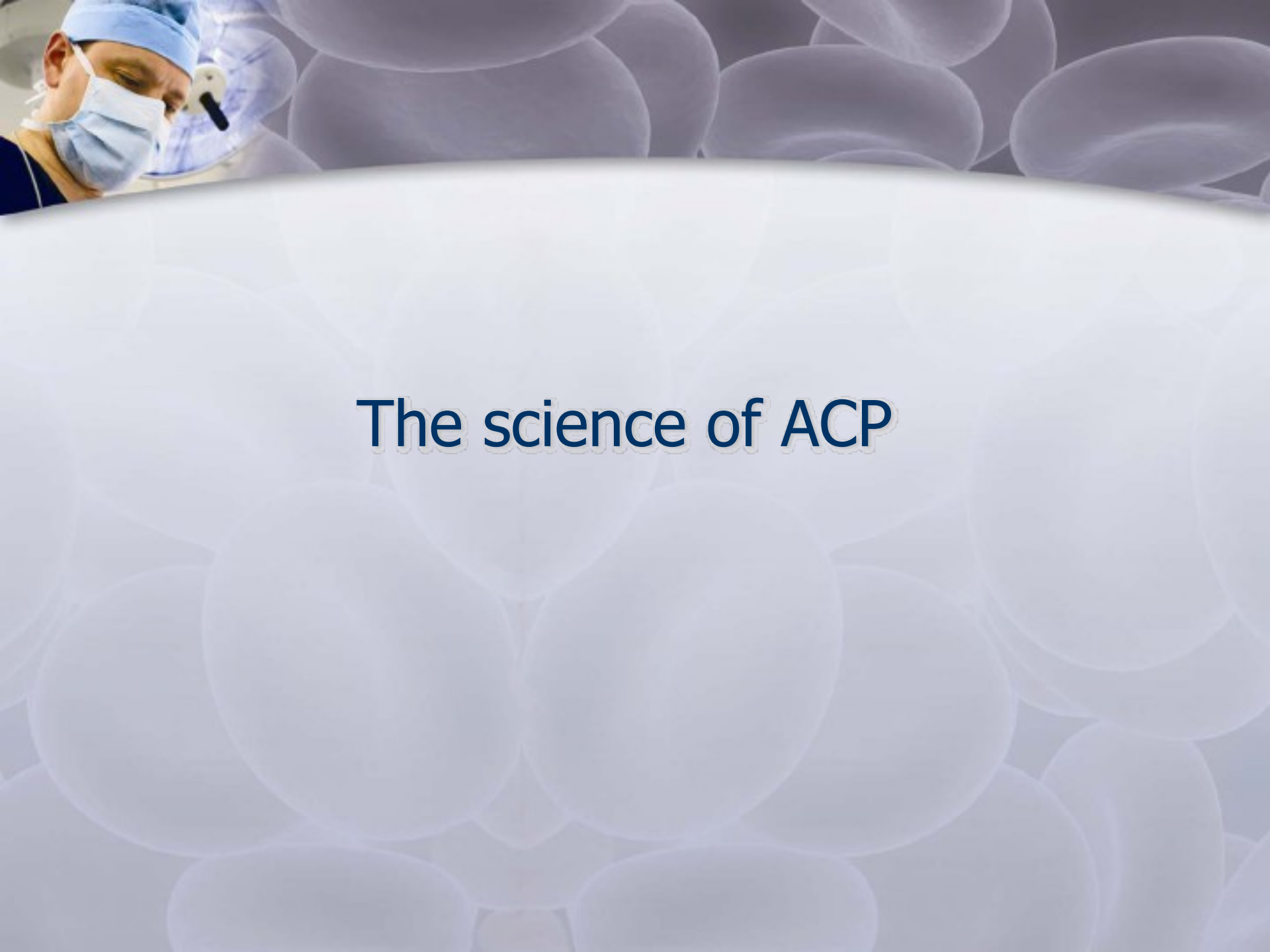
Higher vs. Lower Concentration

- Anitua's definition of platelet concentration
 - Whole blood baseline – $1.5 - 2 \times 10^5$ platelets / μL
 - **Minimum 3×10^5 platelets / μL – 2X PRP**
- **2 - 3X ACP concentration modulates healing**
 - Other systems use similar concentrations
 - Plasma rich in growth factors (PRGF) – Spain
 - Clinaseal centrifuge - US
 - Nahita centrifuge - Spain



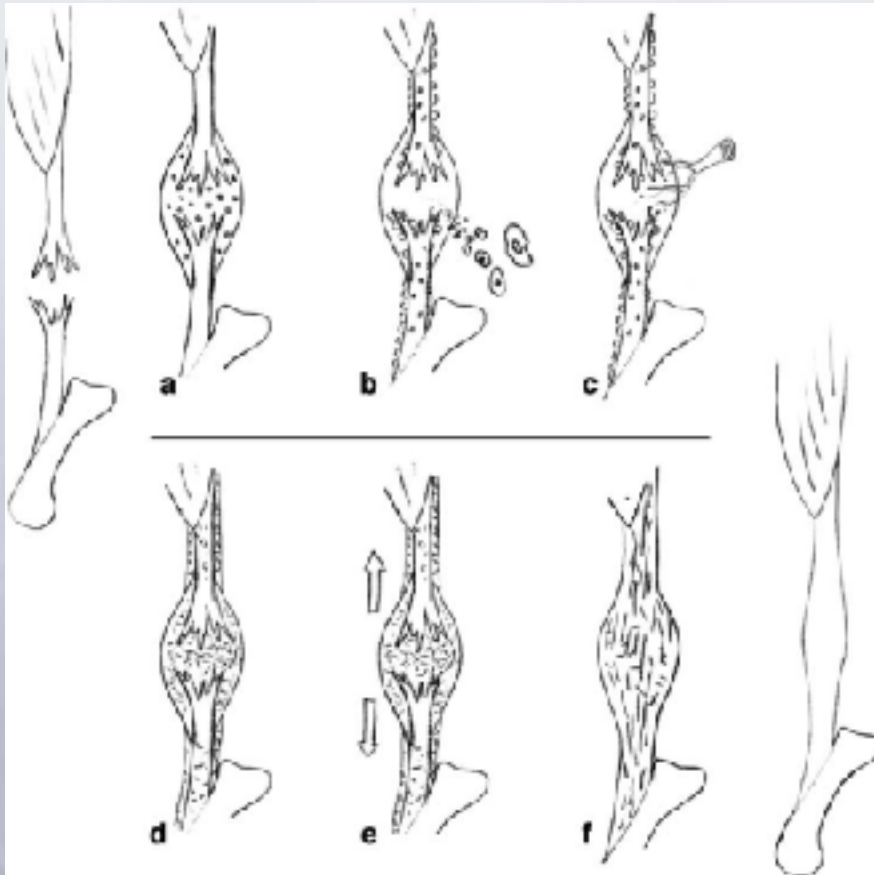
Higher vs. Lower Concentration

- Marx's definition of PRP concentration
 - Whole blood baseline – $1.5 - 2 \times 10^5$ platelets / μL
 - **Minimum 1×10^6 platelets / μL - 5X PRP**



The science of ACP

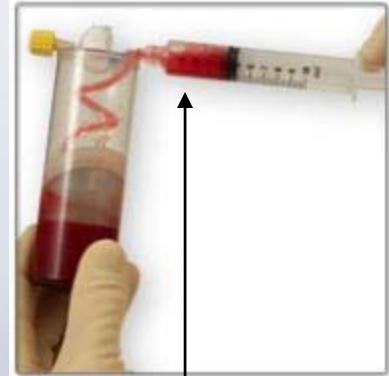
Healing Cascade



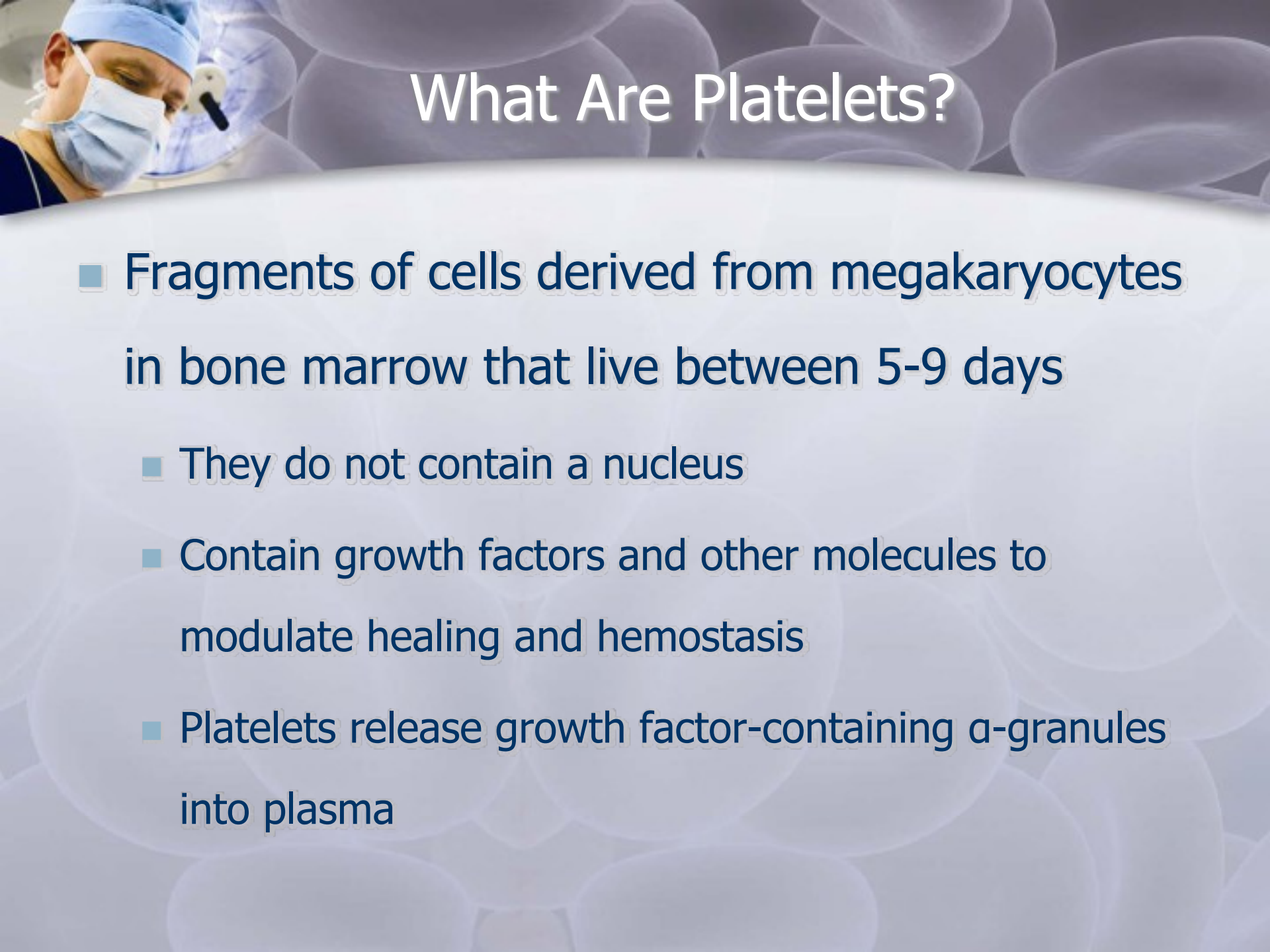
- a – Hematoma with platelet activation
- b – Invasion of cells and proliferation of paratenon
- c – Vascular and neuronal ingrowth
- d – Loose collagenous callus formation
- e – Mechanical stimulation
- f – Maturation and remodeling

Most systems concentrate RBCs and WBCs – why this is bad

- Scott A *et al.* What do we mean by the term "inflammation"? A contemporary basic science update for sports medicine. Br J Sports Med 2004; 38(3): 372-80.
 - **Activated WBCs release MMPs that degrade matrix and reactive oxygen species that destroy anything close by - can lead to inflammation**
- Jiang N *et al.* Respiratory protein-generated reactive oxygen species as an antimicrobial strategy. Nat Immunol 2007; 8(10): 1114-22.
 - **Lysed RBCs release free radicals, similar to WBCs, that destroy anything close by – good for bacteria, not good for platelets**



Why is this
resultant
PRP **RED**?



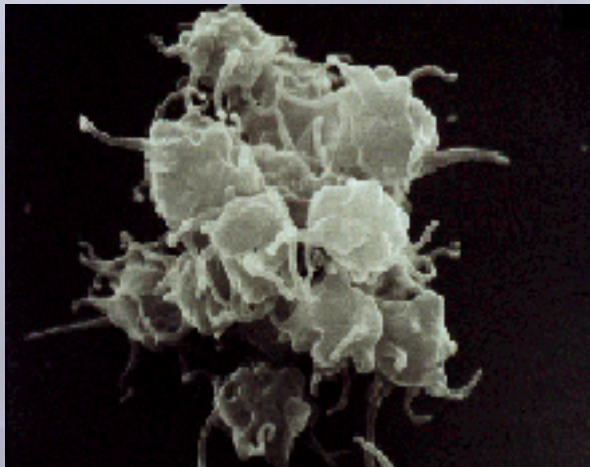
What Are Platelets?

- Fragments of cells derived from megakaryocytes in bone marrow that live between 5-9 days
 - They do not contain a nucleus
 - Contain growth factors and other molecules to modulate healing and hemostasis
 - Platelets release growth factor-containing α -granules into plasma

Platelet Activation



Unactivated platelets



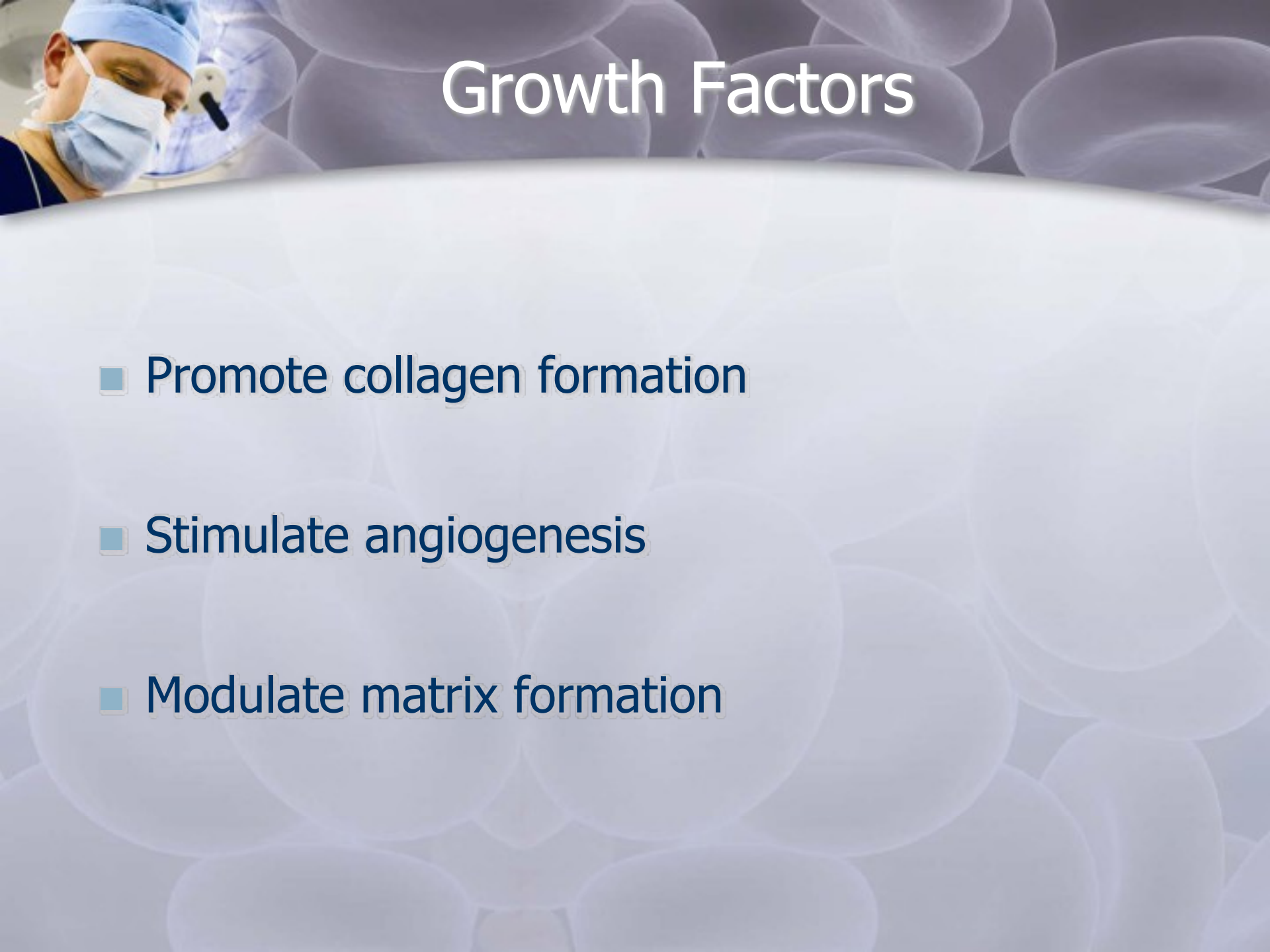
Activated platelets

- Releases growth factors and other cytokines from α -granules
 - Calcium chloride or any other calcium salt
 - Thrombin
- Centrifugation activates platelets, but to much smaller extent



ACD-A

- All PRP systems require the addition of an anticoagulant (ACD-A)
 - These additives drive down the pH and dilute the end product
 - Why is this a problem?
 - Possible solution to add sodium bicarbonate
- When to use or not to use?
 - <30 minutes - no need
 - >30 minutes - use ACD-A, will stay for 4 hours



Growth Factors

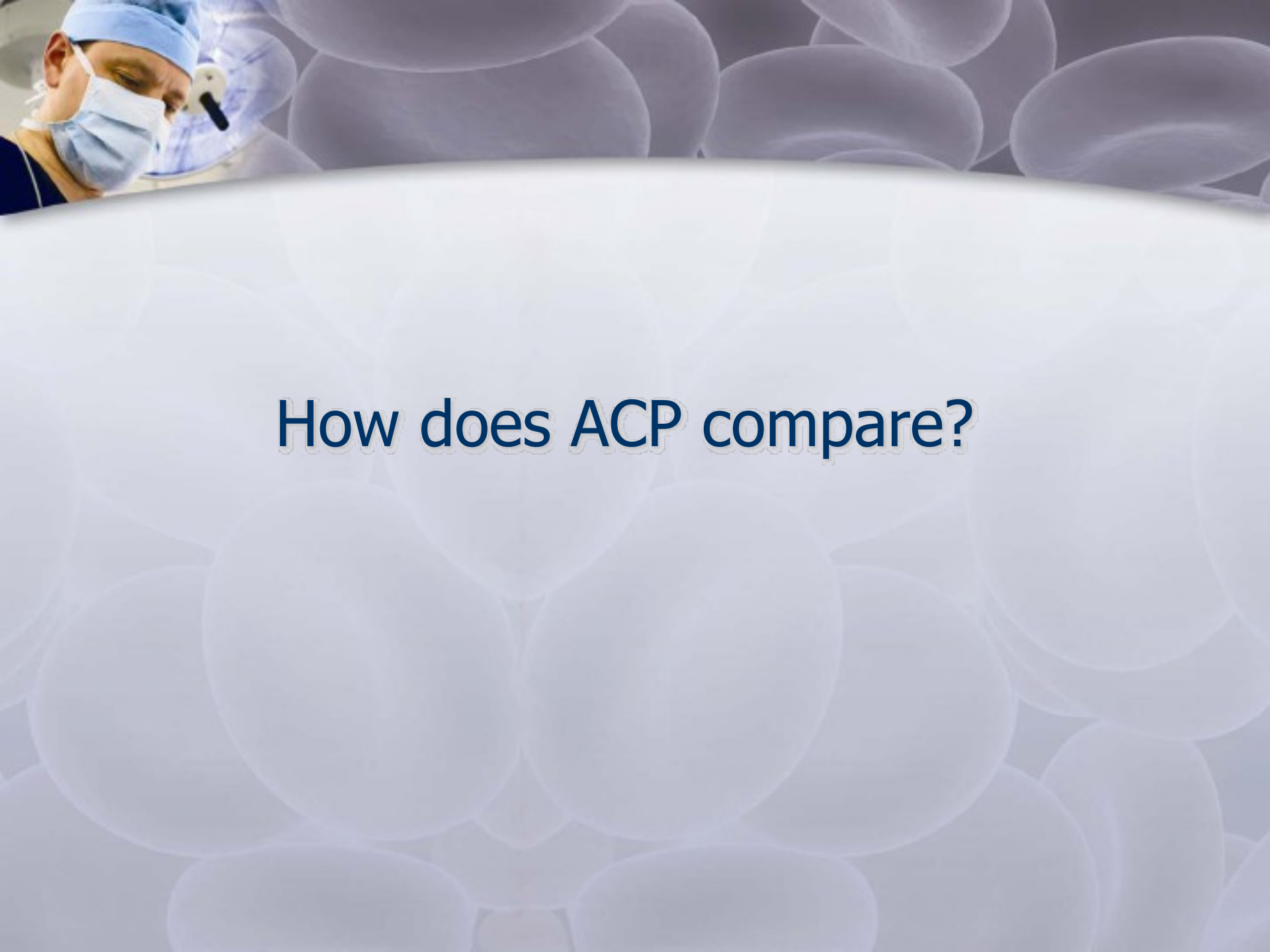
- Promote collagen formation
- Stimulate angiogenesis
- Modulate matrix formation



Growth Factors in ACP

Growth Factor	Phase in Which Most Active	Functions
IGF-1	Inflammation, proliferation	Promotes proliferation and migration of cells, stimulates matrix production
TGF- β	Inflammation	Regulates cell migration, proteinase expression, fibronectin binding interactions, termination of cell proliferation, stimulation of collagen production
VEGF	Proliferation, remodeling	Promotes angiogenesis
PDGF	Proliferation, remodeling	Regulates protein and DNA synthesis at injury site, regulates expression of other growth factors
bFGF	Proliferation, remodeling	Promotes cellular migration, angiogenesis
EGF	Proliferation, remodeling	Stimulates proliferation and differentiation of epidermal cells, stimulates angiogenesis

Adapted from Molloy *et al*, Sports Med 2003; 33(5): 381-94



How does ACP compare?



Comparison of ACP vs. PRP

	Arthrex ACP	Other PRP Systems
Volume drawn	10 mL	60-120 mL
Is anticoagulant (ACD-A) necessary?	No: if ACP is used within 30 minutes	Yes
Centrifugation steps	1X	1-2X
Centrifugation time	5 min	15-30 min
Does it concentrate red and white blood cells?	No: < whole blood levels	Yes: >= whole blood levels
Cost	\$150	\$350-900
Harvest volume	2-5 mL	6-10 mL
Growth Factor Concentration	~5X -25X	~5-10X
Mode of action	Growth factors released by platelets, mixed with proteins and electrolytes in plasma	Growth factors released by platelets
Platelet concentration/ μ L	300,000 – 500,000	500,000 – 1,000,000



Comparison to Other Systems

Device Name	IGF-1 Increase	TGF- β Increase	VEGF Increase	PDGF-AB Increase	PDGF-BB Increase	EGF Increase
<i>Arthrex ACP</i>	<i>1X</i>	<i>4X</i>	<i>11X</i>	<i>25X</i>	<i>6X</i>	<i>5X</i>
¹ Biomet GPS™	1X	3.6X	6.2X	N/A	5.1X	3.9X
² Harvest® SmartPrep2™	N/A	4.4X	4.4X	4.4X	N/A	4.4X
³ DePuy Symphony II	N/A	3-6X	2-3X	4-5X	N/A	4-6X
⁴ Medtronic Magellan™	N/A	4-6X	3-6X	6-10X	N/A	8-10X
⁵ Cascade Fibrinet	5-10X*	5-10X*	5-10X*	N/A	5-10X*	5-10X*

1. GPS – From Eppley *et al*, Plast Reconstr Surg, 114: 1502, 2004 and Biomet website
 2. SmartPrep2 – From Harvest Technologies brochure
 3. Symphony II – From DePuy White Paper
 4. Magellan – From Medtronic brochure
 5. Fibrinet – From Cascade Medical brochure, presentation, and White Papers
- * Fibrinet growth factor levels monitored over a 7 day period - all others were baseline



Our Competition's Story

- What are they going to say?
 - Lower platelet concentration levels
 - The product from ACP is PPP
 - Not true!
 - Delivery method: To clot or not?
 - Using thrombin = immediate release of GF
 - Kit design (Micromedics' FibriJet parts)
 - Where are the studies?



Key Features and Benefits

- Affordable
- Easy to use and short procedure time
- Requires less blood than conventional PRP systems
- Anticoagulant not necessary in all cases
 - Less dilution
 - More natural pH
- Reduced level of RBC and WBCs vs. baseline
- Similar GF concentration to other systems



PRP Gelling Agent Solution

- Gelling agent powder – 4800-5000 IUs (International Units)
- Gelling agent liquid – 5 mL of 10% solution
- Mix 1000 IUs of gelling agent powder with 1 mL of gelling agent liquid (1000 IUs/mL ratio of powder/liquid)
 - 5000 IUs of powder to 5 mL of liquid
 - 10000 IUs of powder to 10 mL of liquid
 - This creates the gelling agent solution
- Mix 1 mL of PRP with 0.1 mL of gelling agent solution to create a clot/gel (10:1 ratio of PRP/gelling agent solution)
 - Can be prepared with syringe mixer system or in sterile cup
 - If clot prepared in sterile cup, transfer clot to dry sterile cup after forming to make it more solid



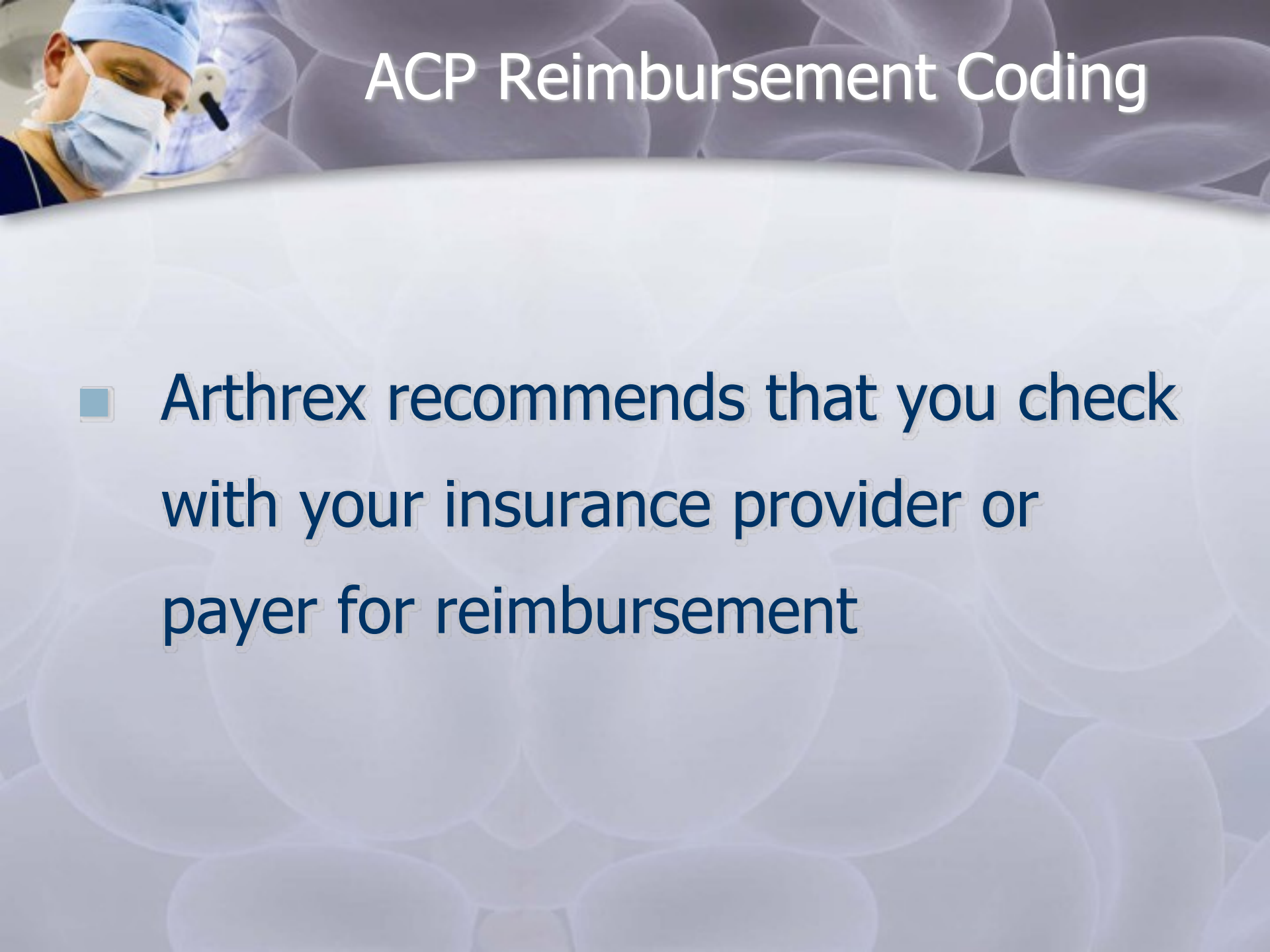
Capital Program

■ Two Program Options

- If the capital equipment is placed and disposables \$200
 - Minimum purchase \$8,000 in disposables over 12 months
 - Disposables convert to \$150 after minimum purchase is met
- If the capital equipment is placed and disposables \$150
 - Minimum purchase \$8,000 in disposables over 12 months
 - Reduced commissions until minimum purchase is met
- Capital Equipment Cost \$7K
- Distributorship may purchase capital under conventional Arthrex capital agreement

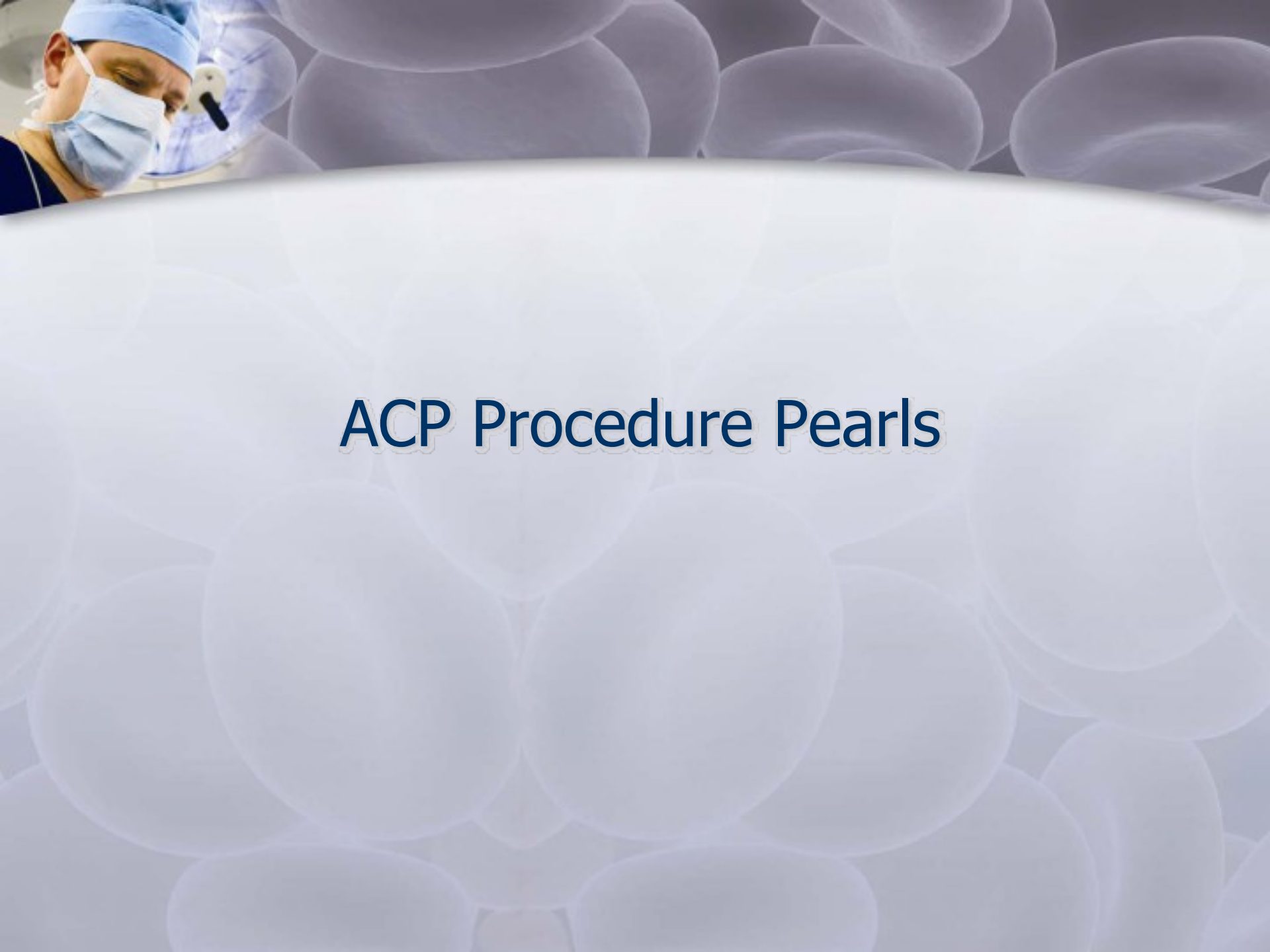


Coding / Reimbursement



ACP Reimbursement Coding

- Arthrex recommends that you check with your insurance provider or payer for reimbursement



ACP Procedure Pearls



ACP Pearls

- Prior to the case
 - Prime the syringe
 - Make sure the inner syringe is screwed in tightly
- Phlebotomy
 - Blood draw should be performed with a 18-20G butterfly needle
 - Draw blood **SLOWLY** (1cc / 2 seconds)
- Separate ACP as soon as possible after blood draw
- If using within 30 min of blood draw, do not use ACD-A
- ACP with ACD-A can stay up to 4 hours at room temperature before use, but use is recommended within 2 hours



**CENTRIFUGE
UNPACKING & SETUP**

**EVERY CENTRIFUGE
SHOULD HAVE A
SHIPPING INSERT LIKE
THIS PACKED WITH IT,
USUALLY INSIDE THE
CENTRIFUGE CHAMBER**

ROTOFIX 32A

ACHTUNG

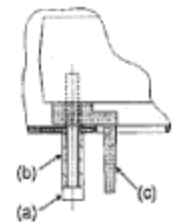
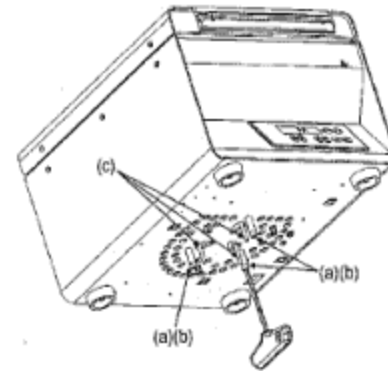
Transportsicherung unbedingt entfernen

CAUTION

Remove transport protection unconditionally

ATTENTION

Retirer impérativement la protection de transport



1. Die 3 Schrauben (a) und Abstandshülsen (b) entfernen.
2. Die Transportsicherung (c) 3x entfernen.

1. Remove the 3 screws (a) and distance sleeves (b).
2. Remove the transport protection (c) 3x.

1. Enlever les 3 vis (a) et ses capuchons (b).
2. Enlever les protections de transport (c) 3x.

Achtung: Die Transportsicherung muss vor jedem Transport eingebaut werden.

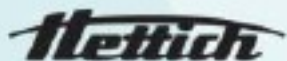
Caution: The transport protection has to be installed before each transportation.

Attention: La protection de transport doit être installée avant chaque transport.

INCLUDED WITH ROTOFIX 32A

- *SHORT T-HANDLE (E613)*
- *GREASE FOR ROTOR (4051)*
- *13 AMP POWER CABLE (6083)*
- *PLASTIC EMERGENCY OPEN PIN (E2914)*
- *TWO SPARE FUSES (E914)*
- *OPERATOR'S MANUAL*
- *SHIPPING INSERT*

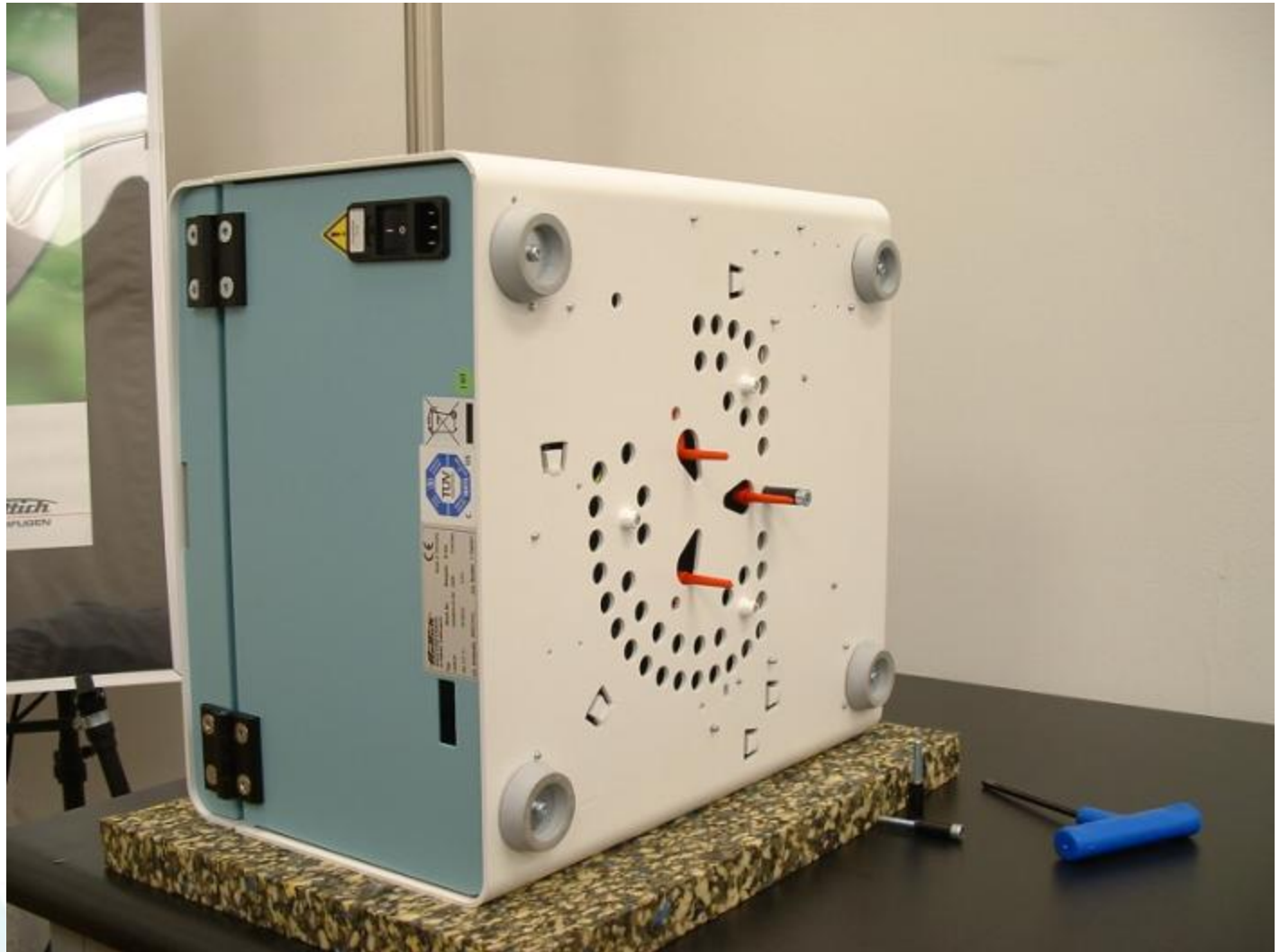
** Numbers in () are part numbers*



REMOVE AND SAVE THE TRANSPORT BOLTS BY UNSCREWING THEM WITH THE T-HANDLE TOOL INCLUDED IN THE ACCESSORY PACK



UNPACKING & SETUP



UNPACKING & SETUP

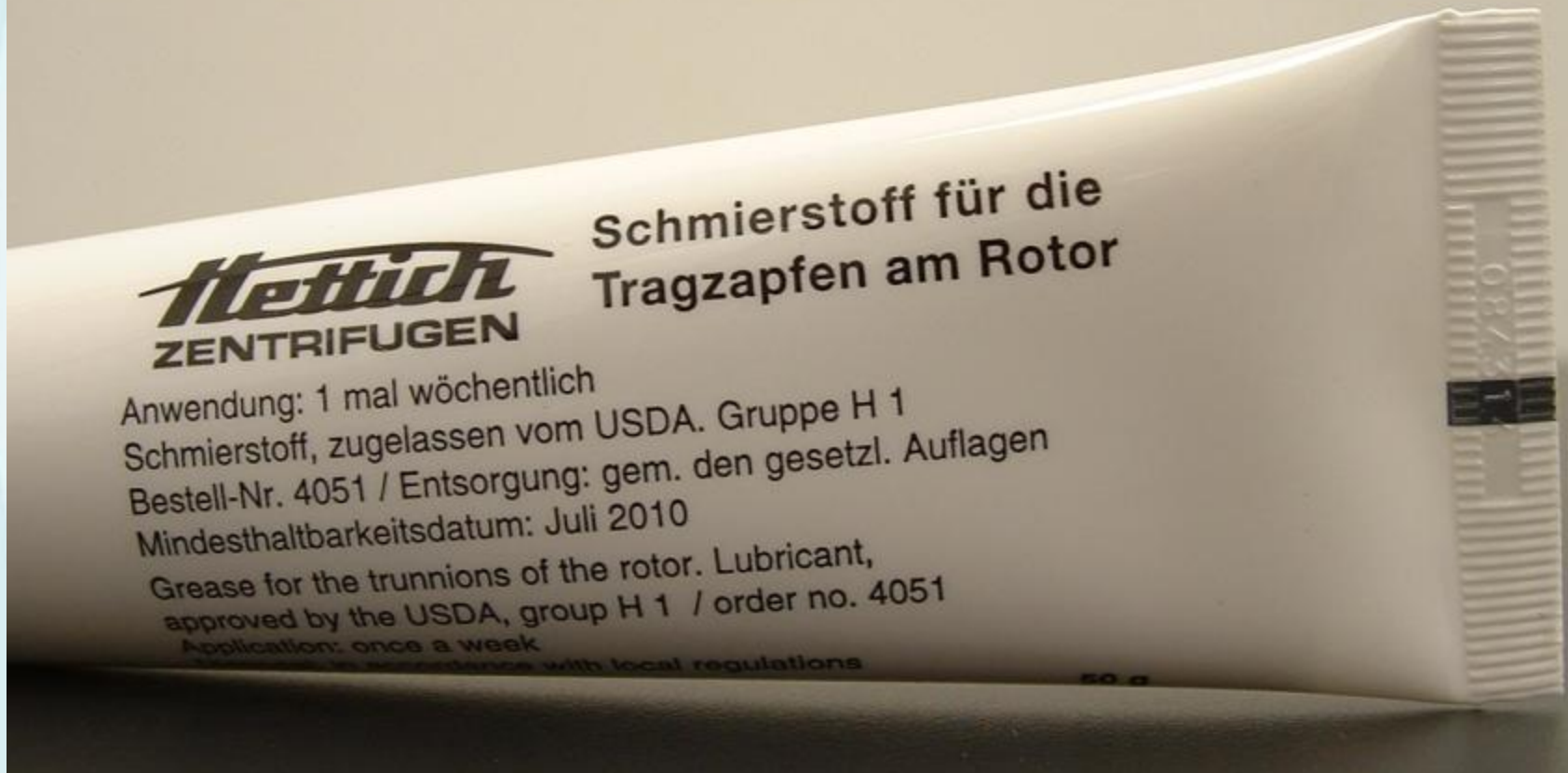
WHEN INSTALLING THE ROTOR, APPLY A THIN COATING OF HETTICH GREASE ON THE MOTOR SHAFT TO PREVENT ROTOR FROM BINDING TO THE SHAFT OVER TIME.

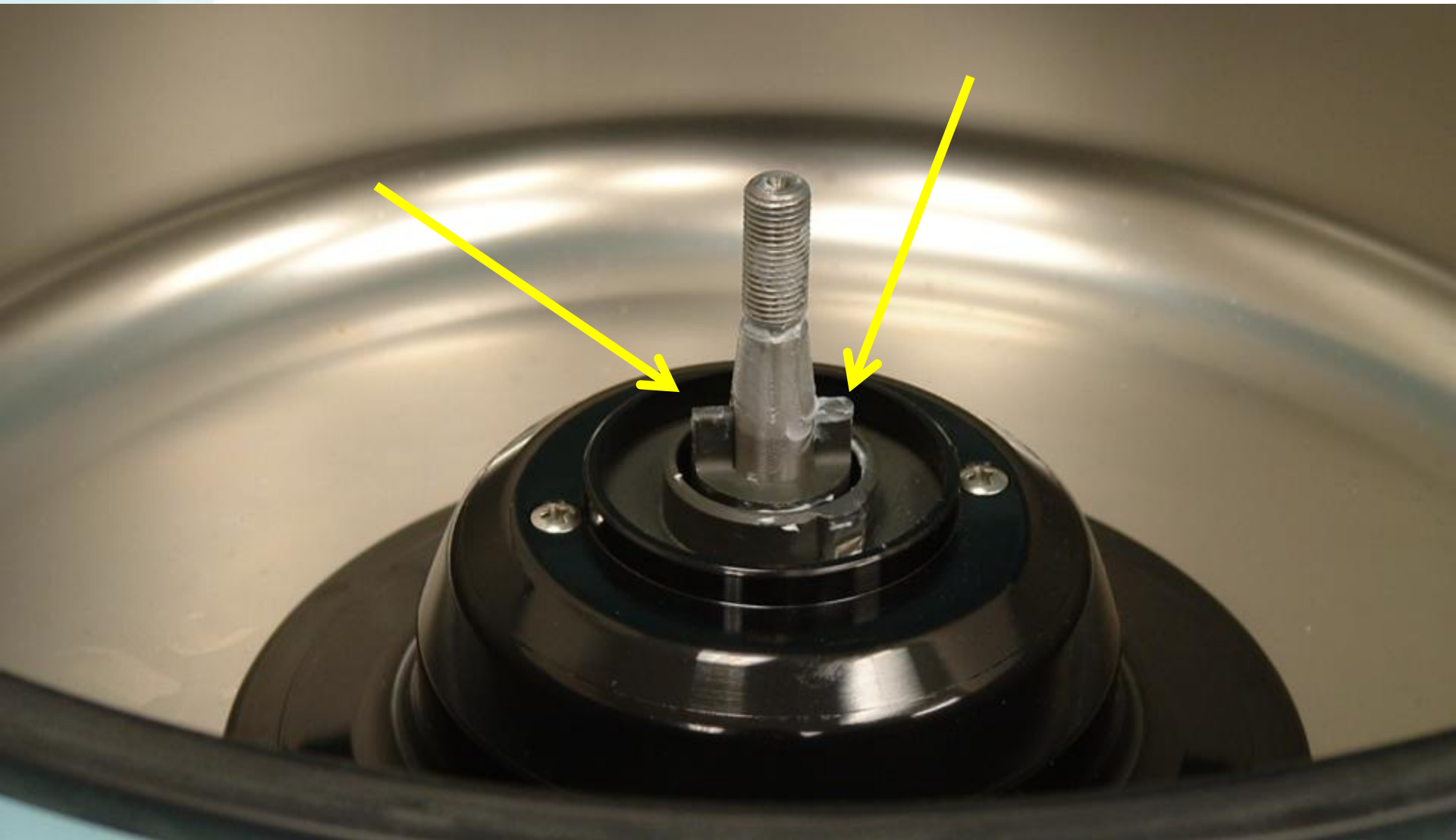


Hettich

UNPACKING & SETUP

PART # 4051

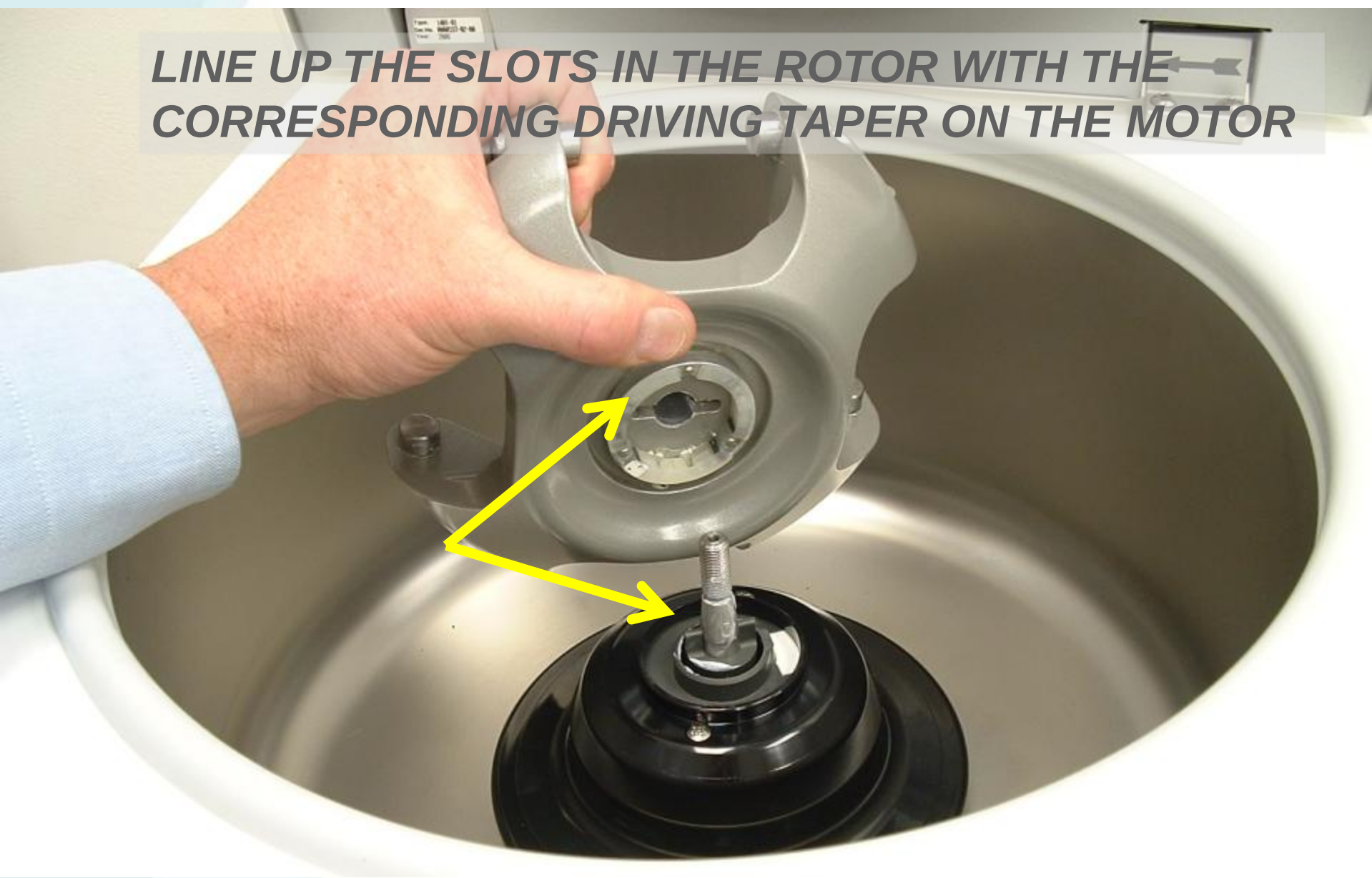




Hettich

UNPACKING & SETUP

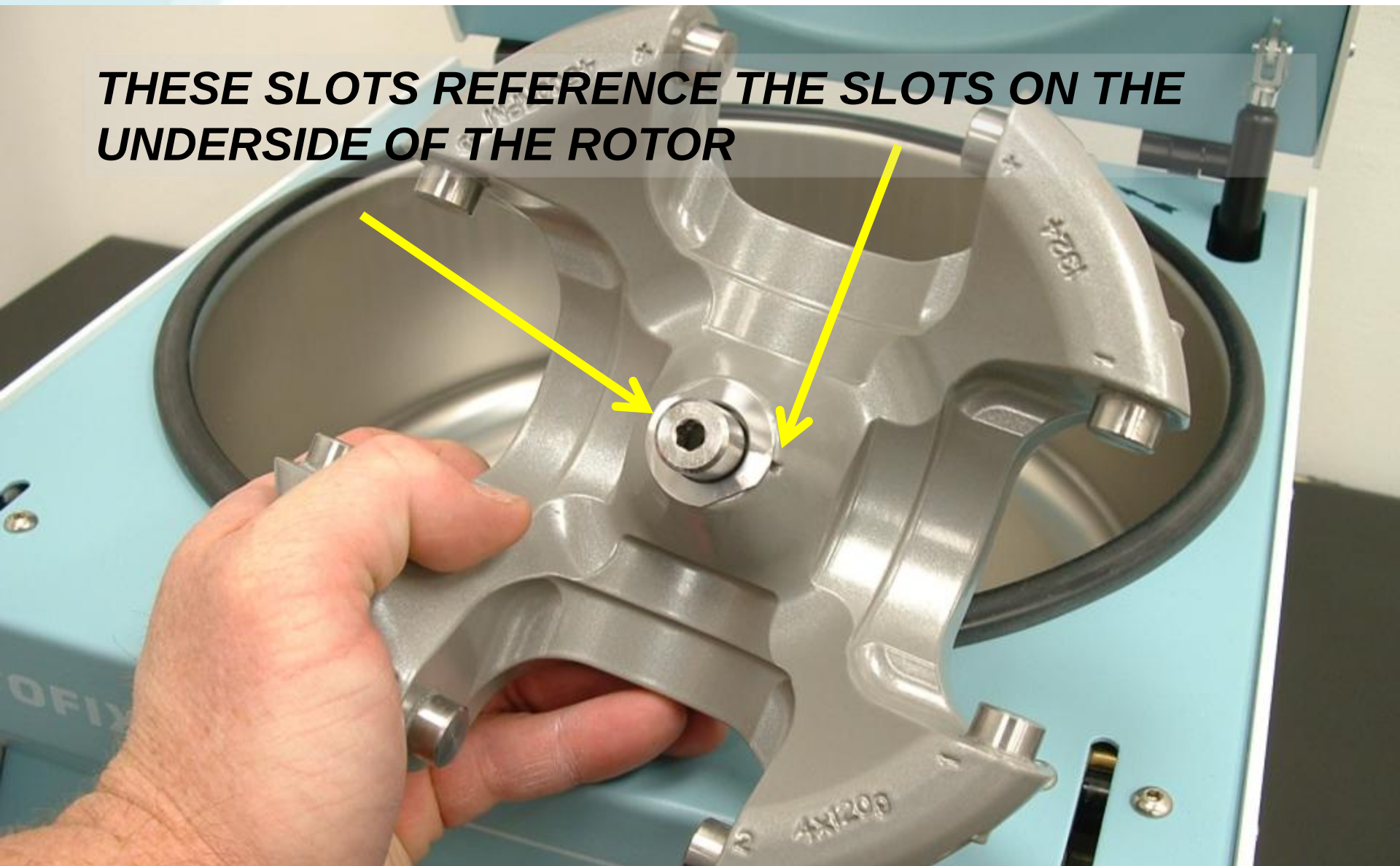
**LINE UP THE SLOTS IN THE ROTOR WITH THE
CORRESPONDING DRIVING TAPER ON THE MOTOR**



Hettich

UNPACKING & SETUP

**THESE SLOTS REFERENCE THE SLOTS ON THE
UNDERSIDE OF THE ROTOR**

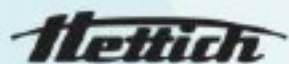


Hettich

UNPACKING & SETUP



**USE THE T-HANDLE TOOL PROVIDED IN THE
ACCESSORY PACK TO TIGHTEN DON THE ROTOR**



UNPACKING & SETUP

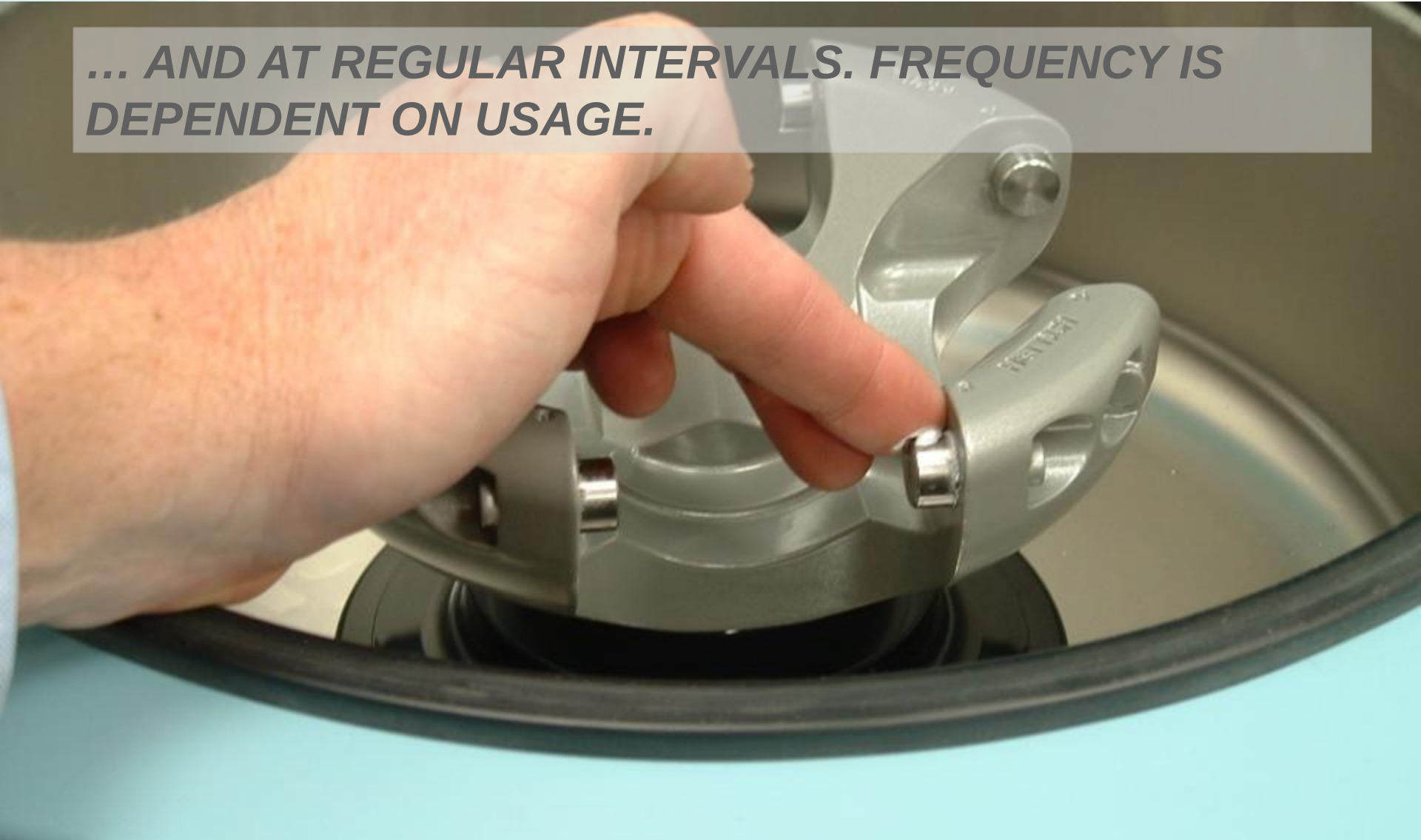
GREASE EACH TRUNNION AT INITIAL SET-UP...



Hettich

UNPACKING & SETUP

***... AND AT REGULAR INTERVALS. FREQUENCY IS
DEPENDENT ON USAGE.***



Hettich

UNPACKING & SETUP

Bucket installation

- MAKE SURE TRUNNIONS ARE GREASED
- ALL FOUR BUCKETS MUST BE INSTALLED

PROGRAMMING THE ROTOFIX 32A

- **USE THE UP AND DOWN BUTTONS TO SET THE TIME AND SPEED**
- **TIME IS IN MINUTES**
- **SPEED IS ____*100
(EXAMPLE:
15=1500RPM)**



SUMMARY SET UP STEPS

- **UNPACKAGE CENTRIFUGE**
- **PLUG IN AND TURN ON**
- **OPEN LID AND REMOVE SHIPPING INSERT**
- **REMOVE AND SAVE TRANSPORT BOLTS AND RED SPACERS**
- **GREASE MOTOR SHAFT AND INSTALL ROTOR**
- **GREASE TRUNNIONS AND INSTALL BUCKETS**
- **BEGIN CENTRIFUGATION**

COMMON ERRORS

- **ERROR CODE -1- (tachometer error)**
- This is explained in the manual on page 28

Clearing procedure

- *Wait for rotor stand still (120 seconds after rotor stops)*
- *Turn the power off and wait 10 seconds*
- *Turn the power on*
- *Check to make sure rotor is installed properly before spinning*

COMMON ERRORS

- **ERROR CODE -2- (mains interrupt)**
- **This is explained in the manual on page 28**

Clearing procedure

- ***Wait for rotor stand still***
- ***Open the lid***
- ***Press the start button***

COMMON ERRORS

- **ERROR CODE -d- (lid lock error)**
- **This is explained in the manual on page 28**

Clearing procedure

- ***Wait for rotor stand still***
- ***Turn off the power and wait 10 seconds***
- ***Turn on the power***

COMMON ERRORS

- **ERROR CODE -F- (tachometer error)**
- **This is explained in the manual on page 28**

Clearing procedure

- ***Wait for rotor stand still***
- ***Turn off power wait 10 seconds***
- ***Turn on power***
- ***Check to make sure rotor is installed properly before spinning***